



AVENTUS HEALTH, LLC
1341 Sundial Point,
Winter Springs, FL 32708
Ph# (321) 283-6180
Fx# (321) 283-6175

Laboratory Director:
Dr. Michelle Taylor, DO
CLIA ID#:
10D2119613

COMPLETE

Doctor:	Hightower, Erica (789)	Patient:	Jangaon, Estella	Acc #:	1000130626
Address:	10490 Huffmeister Rd, Suite B.	Birth:	04/09/1963	Home Phone:	(281)373-5940
CSZ:	Houston, TX 77065	Age:	55	Collection Date:	06/12/2018 07:30 AM
		Gender:	Female	Received Date:	06/14/2018 10:25 AM

Result Comment:

Test Name	In Range	Outside Range	Reference Range	Units
Send Out Core Lab Testing	See Attached Report			

Trinity Hospital

315 Knapp Street

Wolf Point, Montana 59201

406-653-6500

CLIA ID # 27D0409775

Laboratory Director: Mark Sheiko, MD

Name: JANGAON, ESTELLA

DOB: 4/9/1963 Age: 55 years

Gender: FEMALE

Reference #: NEMHS07278588

Location: TRINITY NEMHS

Physician: Hightower, Erica MD

CBC

Order	Accession	Collected		
CBC no Diff	18-163-4676	6/12/2018 07:30 MDT		
Test	Result	Units	Reference Range	Verified
WBC	6.7	X(10)3/uL	4.1-10.9	6/13/2018 12:18 MDT
RBC	3.92 ^L	X(10)6/uL	4.00-5.20	6/13/2018 12:18 MDT
HGB	11.9 ^L	gm/dL	12.0-16.0	6/13/2018 12:18 MDT
Hct	35.5 ^L	%	36.0-48.0	6/13/2018 12:18 MDT
MCH	30	pg	26-32	6/13/2018 12:18 MDT
MCV	91	fL	80-97	6/13/2018 12:18 MDT
RDW	13.3	%	11.0-15.1	6/13/2018 12:18 MDT
PLT	247	X(10)3/uL	140-440	6/13/2018 12:18 MDT
MCHC	34	gm/dL	32-36	6/13/2018 12:18 MDT
MPV	11.2 ^H	fL	6.0-9.2	6/13/2018 12:18 MDT

General Chemistry

Order	Accession	Collected		
Comp Metabolic	18-163-4676	6/12/2018 07:30 MDT		
Test	Result	Units	Reference Range	Verified
Sodium	143	mEq/L	136-145	6/13/2018 13:03 MDT
Potassium	4.0	mmol/L	3.5-5.1	6/13/2018 13:03 MDT
Chloride	104	mEq/L	98-107	6/13/2018 13:03 MDT
CO2	28	mmol/L	22-30	6/13/2018 13:03 MDT
BUN	18	mg/dL	7-20	6/13/2018 13:03 MDT
Creatinine	0.5 ^L	mg/dL	0.6-1.2	6/13/2018 13:03 MDT
GFR non African American	">60" ^{il}	mL/min/1.73m2		6/13/2018 13:03 MDT
Glucose,Bld	95	mg/dL	70-105	6/13/2018 13:03 MDT
AST	20	Units/L	10-46	6/13/2018 13:03 MDT
ALT	18 ^{il}	Units/L	<=35	6/13/2018 13:03 MDT
Alk Phos	56	Units/L	35-128	6/13/2018 13:03 MDT
Albumin	4.00	gm/dL	3.50-5.20	6/13/2018 13:03 MDT
Protein,Total	7.3	gm/dL	6.2-8.8	6/13/2018 13:03 MDT
T.Bil	0.6	mg/dL	0.2-1.3	6/13/2018 13:03 MDT
Calcium	9.1	mg/dL	8.0-10.3	6/13/2018 13:03 MDT

Interpretive Data

il: GFR non African American
Chronic Kidney Disease less than 60 mL/min/1.73m2

H=High L=Low @=Abnormal C=Critical *=Corrected i=Interpretation f=Comments

Print Date/Time: 6/13/2018 17:09 MDT

Page 1 of 4

RRID: 196080859

Trinity Hospital**315 Knapp Street****Wolf Point, Montana 59201****406-653-6500****CLIA ID # 27D0409775****Laboratory Director: Mark Sheiko, MD****Name: JANGAON, ESTELLA****DOB: 4/9/1963 Age: 55 years****Gender: FEMALE****Reference #: NEMHS07278588****Location: TRINITY NEMHS****Physician: Hightower, Erica MD****General Chemistry****Interpretive Data**

- i1: GFR non African American
Kidney Failure less than 15 mL/min/1.73m2

Estimated GFR is a screening test only. This should not be used for determining medication dosages. GFR is calculated using the MDRD equation and normalized to 1.73m2 body surface area.

- i2: To calculate the African American GFR, multiply the reported result by 1.21.
ALT
New ALT method. Please note Reference Range change.

Order		Accession	Collected	
Ferritin, Plasma		18-163-4676	6/12/2018 07:30 MDT	
Test	Result	Units	Reference Range	Verified
Ferritin	154.3	ng/mL	15.0-200.0	6/13/2018 14:46 MDT

Order		Accession	Collected	
Iron Panel, NEMH		18-163-4676	6/12/2018 07:30 MDT	
Test	Result	Units	Reference Range	Verified
Iron	113.0	ug/dL	40.0-155.0	6/13/2018 13:23 MDT
TIBC	328.0	ug/dL	250.0-370.0	6/13/2018 13:03 MDT
Calculated UIBC	215.0 ^L	ug/dL	255.0-450.0	6/13/2018 13:23 MDT
Transferrin Sat	34.5	%	20.0-50.0	6/13/2018 13:23 MDT

Order		Accession	Collected	
Lipid Panel, Regional		18-163-4676	6/12/2018 07:30 MDT	
Test	Result	Units	Reference Range	Verified
Cholesterol	118 ^L	mg/dL	150-200	6/13/2018 13:03 MDT
Trig	78 ^u	mg/dL	0-199	6/13/2018 13:03 MDT
HDL	46.0	mg/dL	40.0-60.0	6/13/2018 13:03 MDT
Chol/HDL	2.6 ^u		1.0-3.5	6/13/2018 13:03 MDT
LDL	56	mg/dL	0-160	6/13/2018 13:03 MDT

Interpretive Data

- i3: Trig
Triglyceride Normal Range: Less than 150 mg/dL
Borderline High: 150-199 mg/dL
High: 200-499 mg/dL

H=High L=Low @=Abnormal C=Critical *=Corrected I=Interpretation f=Comments

Print Date/Time: 6/13/2018 17:09 MDT

Page 2 of 4

RRID: 196080859

Trinity Hospital**315 Knapp Street****Wolf Point, Montana 59201****406-653-6500****CLIA ID # 27D0409775****Laboratory Director: Mark Sheiko, MD****Name: JANGAON, ESTELLA****DOB: 4/9/1963 Age: 55 years****Gender: FEMALE****Reference #: NEMHS07278588****Location: TRINITY NEMHS****Physician: Hightower, Erica MD****General Chemistry****Interpretive Data**

i3: Trig
Very High: Greater than 500 mg/dL

i4: Chol/HDL
Male:
Below Average: 3.43
Average: 4.97
Very High: 23.39
Female:
Below Average: 3.27
Average: 4.44
High: 7.05
Very High: 11.04

High: 9.55

Order	Accession	Collected
Magnesium, Blood	18-163-4676	6/12/2018 07:30 MDT
Test	Result	Units
Magnesium	1.8	mg/dL
Reference Range	1.6-2.3	Verified
		6/13/2018 13:03 MDT

Order	Accession	Collected
T3 Free	18-163-4676	6/12/2018 07:30 MDT
Test	Result	Units
Free T3	2.4	pg/dL
Reference Range	2.0-4.9	Verified
		6/13/2018 14:46 MDT

Order	Accession	Collected
T4 Free	18-163-4676	6/12/2018 07:30 MDT
Test	Result	Units
T4 Free	1.50	ng/dL
Reference Range	0.75-1.54	Verified
		6/13/2018 14:46 MDT

Order	Accession	Collected
TSH	18-163-4676	6/12/2018 07:30 MDT
Test	Result	Units
TSH	1.65	uIU/mL
Reference Range	0.42-4.00	Verified
		6/13/2018 14:46 MDT

H=High L=Low @=Abnormal C=Critical *=Corrected i=Interpretation f=Comments

Print Date/Time: 6/13/2018 17:09 MDT

Page 3 of 4

RRID: 196080859

Trinity Hospital

315 Knapp Street

Wolf Point, Montana 59201

406-653-6500

CLIA ID # 27D0409775

Laboratory Director: Mark Sheiko, MD

Name: JANGAON, ESTELLA

DOB: 4/9/1963 Age: 55 years

Gender: FEMALE

Reference #: NEMHS07278588

Location: TRINITY NEMHS

Physician: Hightower, Erica MD

Special Chemistry

Order	Accession	Collected		
B12, Vitamin, Blood	18-163-4676	6/12/2018 07:30 MDT		
Test	Result	Units	Reference Range	Verified
Vitamin B12	504.0	pg/mL	230.0-1050.0	6/13/2018 15:39 MDT

H=High

L=Low

@=Abnormal

C=Critical

*=Corrected

i=Interpretation

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Print Date/Time: 6/13/2018 17:09 MDT

Page 4 of 4

RRID: 196080859

Trinity Hospital**315 Knapp Street****Wolf Point, Montana 59201****406-653-6500****CLIA ID # 27D0409775****Laboratory Director: Mark Sheiko, MD****Name: JANGAON, ESTELLA****DOB: 4/9/1963 Age: 55 years****Gender: FEMALE****Reference #: NEMHS07278588****Location: TRINITY NEMHS****Physician: Hightower, Erica MD****Special Chemistry**

Order	Accession	Collected		
Vit D 25-OH	18-163-4677	6/12/2018 07:30 MDT		
Test	Result	Units	Reference Range	Verified
Vit D 25-OH Total	55.8 ¹¹	ng/mL	30.0-100.0	6/13/2018 13:17 MDT

Interpretive Data

il: Vit D 25-OH Total
 Vitamin D sufficient: 30 to 100 ng/mL
 Vitamin D deficiency: <20 ng/mL
 Vitamin D insufficiency: <30 ng/mL
 Vitamin D toxicity: >100 ng/mL

H=High L=Low @=Abnormal C=Critical *=Corrected i=Interpretation f=Comments

Print Date/Time: 6/13/2018 17:09 MDT

Page 1 of 1

RRID: 196080857

Trinity Hospital**315 Knapp Street****Wolf Point, Montana 59201****406-653-6500****CLIA ID # 27D0409775****Laboratory Director: Mark Sheiko, MD****Name: JANGAON, ESTELLA****DOB: 4/9/1963 Age: 55 years****Gender: FEMALE****Reference #: NEMHS07278588****Location: TRINITY NEMHS****Physician: Hightower, Erica MD****General Chemistry**

Order	Accession	Collected		
Hgb A1c	18-163-4678	6/12/2018 07:30 MDT		
Test	Result	Units	Reference Range	Verified
Hem A1c	5.6	%	4.0-6.4	6/13/2018 14:15 MDT
Mean Bld Gluc	114	mg/dL		6/13/2018 14:15 MDT

H=High L=Low @=Abnormal C=Critical *=Corrected i=Interpretation f=Comments**Print Date/Time: 6/13/2018 17:09 MDT****Page 1 of 1****RRID: 196080855**

Trinity Hospital

315 Knapp Street

Wolf Point, Montana 59201

406-653-6500

CLIA ID # 27D0409775

Laboratory Director: Mark Sheiko, MD

Name: JANGAON, ESTELLA

DOB: 4/9/1963 Age: 55 years

Gender: FEMALE

Reference #: NEMHS07278588

Location: TRINITY NEMHS

Physician: Hightower, Erica MD

Urine Chemistry

Order	Accession	Collected
UAlb/Creat Ratio	18-163-4679	6/12/2018 07:30 MDT
Test	Result	Units
Creat, Urine	249.7	mg/dL
Microalb/Cr	20	mg/L
Microalb, Ur	50.4 ^B	mg/L
Reference Range	Verified	
	6/13/2018 15:45 MDT	
<=30	6/13/2018 15:45 MDT	
<=20.0	6/13/2018 15:45 MDT	

H=High

L=Low

@=Abnormal

C=Critical

*=Corrected

I=Interpretation

f=Comments

Print Date/Time: 6/13/2018 17:09 MDT

Page 1 of 1

RRID: 196080851

Trinity Hospital**315 Knapp Street****Wolf Point, Montana 59201****406-653-6500****CLIA ID # 27D0409775****Laboratory Director: Mark Sheiko, MD****Name: JANGAON, ESTELLA****DOB: 4/9/1963 Age: 55 years****Gender: FEMALE****Reference #: NEMHS07278588****Location: TRINITY NEMHS****Physician: Hightower, Erica MD****Urinalysis**

Order		Accession	Collected	
UA Microscopic		18-163-4680	6/12/2018 07:30 MDT	
Test	Result	Units	Reference Range	Verified
UA Micro Exam	See Microscopic			6/13/2018 12:48 MDT
WBC,UA	5-10/HPF @		None Seen	6/13/2018 12:48 MDT
RBC	2-5/HPF		None Seen	6/13/2018 12:48 MDT
Bacteria	Trace		None Seen	6/13/2018 12:48 MDT
Epithelial Cells	Few		None Seen	6/13/2018 12:48 MDT
Mucus	Moderate		None Seen	6/13/2018 12:48 MDT
Amorphous Crys	Many		None Seen	6/13/2018 12:48 MDT
Ca Oxalate Crys	Moderate		None Seen	6/13/2018 12:48 MDT

Order		Accession	Collected	
UA Routine Reflex Culture		18-163-4680	6/12/2018 07:30 MDT	
Test	Result	Units	Reference Range	Verified
Color	Yellow		Yellow	6/13/2018 12:48 MDT
Clarity	Turbid @		Clear	6/13/2018 12:48 MDT
Specific Grav	>=1.030 @		1.003-1.030	6/13/2018 12:48 MDT
pH	5.5		4.5-8.0	6/13/2018 12:48 MDT
Glucose,UA	Negative	mg/dL	Negative	6/13/2018 12:48 MDT
Ketones	Negative		Negative	6/13/2018 12:48 MDT
Bilirubin	Negative			6/13/2018 12:48 MDT
Urobilinogen	0.2	mg/dL	0.2	6/13/2018 12:48 MDT
Protein	30 @	mg/dL	Negative	6/13/2018 12:48 MDT
Blood	Moderate @		Negative	6/13/2018 12:48 MDT
Nitrite	Negative		Negative	6/13/2018 12:48 MDT
Leuk Esterase	Small @		Negative	6/13/2018 12:48 MDT

H=High L=Low @=Abnormal C=Critical *=Corrected i=Interpretation f=Comments

Print Date/Time: 6/13/2018 17:09 MDT

Page 1 of 1

RRID: 196080849

SUMMARY REPORT

Estella Jangam

Gender: Female

Age: 55 (DOB: Apr 19 1963)

Weight: 145 lbs

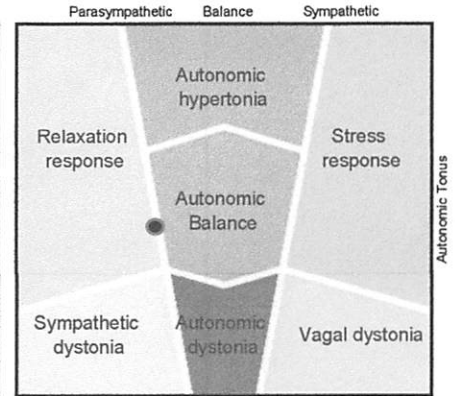
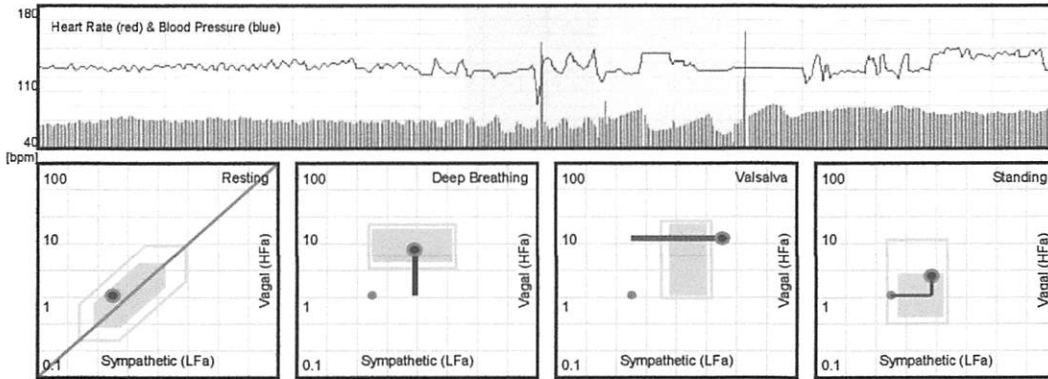
BMI: 26.5

Height: 5 ft 2 in

Physician Only Report

Exam Date: Jun 12 2018 07:50

Autonomic Nervous System Function



	HR	BP	LFa	HFa	SpO2	Comment									
Resting	66	120/57	0.86	1.41	96										
Deep Breathing	62	124/87	2.96	6.24	98	Normal Parasympathetic response to stimulation;									
Valsalva	64	117/79	11.69	8.94	97	Hyperactive sympathetic response to stimulation, this could be cause for potential hypertension;									
Standing	76	124/93	2.75	2.6	93	Normal ANS response to stimulation; Enhanced cardiovascular tolerance. The autonomic regulation of cardiovascular functions provides enormous ability to tolerate physical exertions and normal adaptation to their long-term effects. Typically this is a sign of an absence of physical exhaustion, overtraining or any cardiovascular health conditions. This is a typical pattern found in a physically active, healthy individual, without any evident signs of physical exhaustion, overtraining or other conditions altering the regulatory functions of the autonomic nervous system or the cardiovascular response to it;									
			Score	Units	Norms		Score	Units	Norms		Score	Units	Norms		
Autonomic Activity			55	ms2	40-80	Autonomic Balance		3	ratio	-3 - 3	E/I Ratio		2.24	ratio	>1.1
Valsalva Ratio			1.4	ratio	>1.15	30:15 Ratio		2.08	ratio	>1.1	Functional Age		58	years	< 57
	Score	Norms	Description					Comments							
MSI	-0.38	< 1.0	MSI is an indicator of accumulative mental stress. Mental stress can be caused by situations one has to cope with daily and the emotional states that result. The way in which a person copes with stressful events is a significant contributing					The score is average. It indicates low mental stress. Stress in the recent past has had little effect on your mental health. The negative effects of mental stress are limited.							
PSI	0.18	< 1.0	PSI is an indicator of accumulative chronic physical stress. Physical stress can be caused by long-term exposure to negative factors in one's life or physical illness. The way in which a person copes with physical stressors is a significant...					The score is average. It indicates low physical stress. Stress in the recent past has had little or no long-term effect on your physical balance. The negative effects of physical stress are limited.							

Cardiovascular Function

	Score	Norms	Description	Comments					
EEI	0.3	0.3 - 0.7	EEI is an indicator for left ventricle ejection power and elasticity of large arteries.	Possible reduced left ventricular ejection fraction / atherosclerosis. Possible causes: hypertension...					
DDI	0.42	0.3 - 0.7	DDI indicates the contractility, tension and stiffness in the small arteries.	Normal Blood Circulation					
DEI	0.35	0.3 - 0.7	DEI represents the reflection of arterial elasticity and blood flow in the venous system.	Borderline Normal Blood Circulation					
AI	0.26	< -0.7	Augmentation Index (AI) is a useful marker for cardiac risk. AI increases with age and a	AI is a measure of arterial stiffness and it provides general information about the arteries. AI is					
ABI	1.21	1 - 1.4	Right Ankle/Brachial Index	Normal range					
TBI	1.11	> 0.75	Right Toe/Brachial Index	Normal range					
		Score	Units	Norms			Score	Units	Norms
Reflection Index		0.7		.65-.85	Stiffness Index		11.67	m/s	< 8.0
Cardiac Output		3.69	l/min	4.0-8.0	Mean Arterial Pressure		84	mmHg	70-110
C1		7.41	m/mmHg	> 10.0	C2		6.77	m/mmHg	> 6.0
Ventricular Extrasystole		0		< 1	Atrial Extrasystole		0		< 1
QRS		66	ms	60-120	QTc		304	ms	350-460
					Stroke Volume		56	ml	55-100
					Blood Volume		3.75	l	3-5
					DPTI/SPTI		0.58	ratio	
					Artifacts		0		< 1
					Body Mass Index		26.5		19-25

Sudomotor Function

	Score	Norms	Description	Comments
Sudomotor	40	< 50	Risk for sudomotor autonomic neuropathy. Sweat Gland C-Fiber density and function	No clear sign of Sudomotor dysfunction.

Physician's Notes:

All results and analysis should be considered in the context of persons/candidate's case history, symptoms, diagnosis, current medications, treatment plans and therapies. Final diagnosis is the sole responsibility of the licensed medical practitioner after persons examination, lab tests and/or other clinical findings as necessary.

Printed: Jun 12 2018 08:07

CARDIAC AUTONOMIC NEUROPATHY ASSESSMENT

Estella Jangam

Gender: Female

Age: 55 (DOB: Apr 19 1963)

Weight: 145 lbs

BMI: 26.5

Height: 5 ft 2 in

Physician Only Report

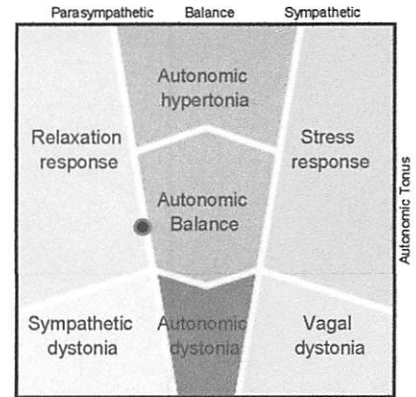
Exam Date: Jun 12 2018 07:50

AUTONOMIC ACTIVITY



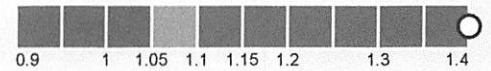
The sympathetic nervous system activates the "fight-or-flight" responses. It activates the glands and organs that defend the body against attack. The sympathetic nervous system uses up energy and depletes the body. The parasympathetic nervous system controls the "rest and digest" responses. The parasympathetic nervous system, when activated by rest, relaxation and happy thoughts, is essential for balanced living and for all healing.

AUTONOMIC BALANCE



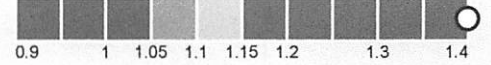
E/I RATIO: 2.24

Expiration/Inspiration Ratio is a marker of cardiovagal response.



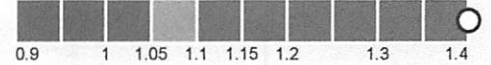
VALSALVA RATIO: 1.4

Marker of baroreceptor sensitivity response.



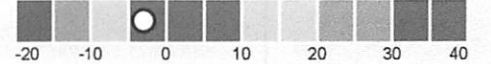
30/15 RATIO: 2.08

Heart rate change during standing or tilt at 30 and 15 second.



Systolic Pressure Response to Standing: -3 mmHg

Marker of sympathetic adrenergic function.



Parasympathetic Response to Deep Breathing

Normal Parasympathetic response to stimulation;

Sympathetic Response to Valsalva Maneuver

Hyperactive sympathetic response to stimulation, this could be cause for potential hypertension;

Active Standing

Normal ANS response to stimulation; Enhanced cardiovascular tolerance. The autonomic regulation of cardiovascular functions provides enormous ability to tolerate physical exertions and normal adaptation to their long-term effects. Typically this is a sign of an absence of physical exhaustion, overtraining or any cardiovascular health conditions. This is a typical pattern found in a physically active, healthy individual, without any evident signs of physical exhaustion, overtraining or other conditions altering the regulatory functions of the autonomic nervous system or the cardiovascular response to it;

Physician's Notes:

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Printed: Jun 12 2018 08:07

AUTONOMIC REGULATION ASSESSMENT

Estella Jangam

Gender: Female

Age: 55 (DOB: Apr 19 1963)

Weight: 145 lbs

BMI: 26.5

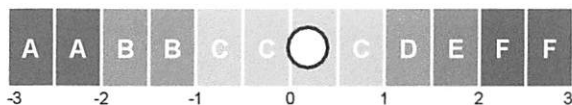
Height: 5 ft 2 in

Physician Only Report

Exam Date: Jun 12 2018 07:50

AUTONOMIC PHYSICAL DYSREGULATION (PSI) = 0.18

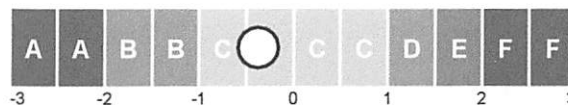
PSI is an indicator of accumulative chronic physical stress. Physical stress can be caused by long-term exposure to negative factors in one's life or physical illness. The way in which a person copes with physical stressors is a significant contributing factor to this process. When the stress becomes too great, the body will show signs of distress.



The score is average. It indicates low physical stress. Stress in the recent past has had little or no long-term effect on your physical balance. The negative effects of physical stress are limited.

ACCUMULATIVE MENTAL STRESS (MSI) = -0.38

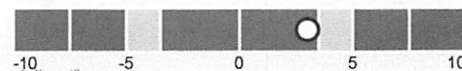
MSI is an indicator of accumulative mental stress. Mental stress can be caused by situations one has to cope with daily and the emotional states that result. The way in which a person copes with stressful events is a significant contributing factor to this process. When the stress becomes too great, the body will show signs of distress.



The score is average. It indicates low mental stress. Stress in the recent past has had little effect on your mental-health. The negative effects of mental stress are limited.

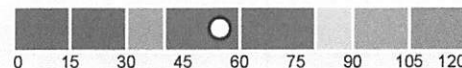
Autonomic Balance: 3

The sympathetic nervous system activates the "fight-or-flight" responses. The parasympathetic nervous system controls the "rest and digest" responses.



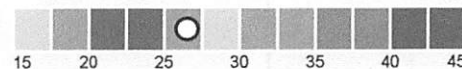
Total Autonomic Activity: 55

Marker of the overall ANS activity at rest.



Body Mass Index (BMI): 26.5

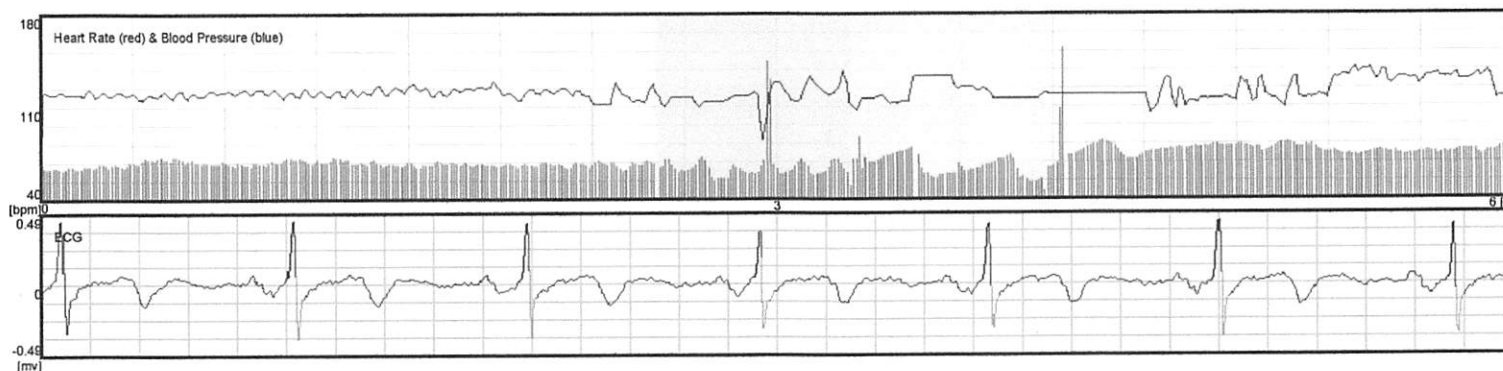
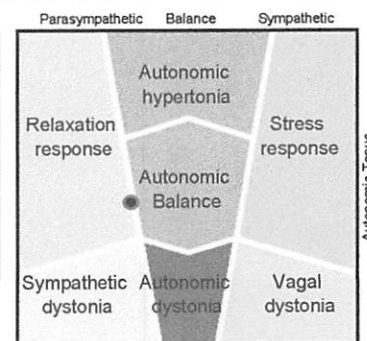
Overweight - prone to health risks



Body mass index is defined as the individual's body weight divided by the square of his or her height. The body mass index can be used to identify if you are overweight.

	Score	Units	Norms		Score	Units	Norms
LFa	0.86	ms2	0.5-4	HFa	1.41	ms2	0.5-4
Blood Pressure	120/57	mmHg	<120	SpO2	96	%	>94
BPSys Response to Standing	-3	mmHg	<10	Functional Age	58	years	< 57
Health Risk Factor	42	%	< 50	SDNN	29	ms	> 40
HeartRate	66	bpm	< 90	Ventricular Extrasystole	0		< 1
Atrial Extrasystole	0		< 1	Artifacts	0		< 1

A - Normal **B** - Borderline Normal **C** - Moderate **D** - Borderline Abnormal **E** - Abnormal **F** - Severe



Physician's Notes:

All results and analysis should be considered in the context of persons/candidate's case history, symptoms, diagnosis, current medications, treatment plans and therapies. Final diagnosis is the sole responsibility of the licensed medical practitioner after persons examination, lab tests and/or other clinical findings as necessary.

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PERIPHERAL ARTERY DISEASE ASSESSMENT

Estella Jangam

Gender: Female

Age: 55 (DOB: Apr 19 1963)

Weight: 145 lbs

BMI: 26.5

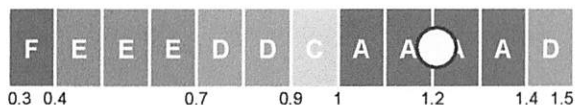
Height: 5 ft 2 in

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Exam Date: Jun 12 2018 07:50

Right ANKLE/BRACHIAL INDEX (ABI) = 1.21

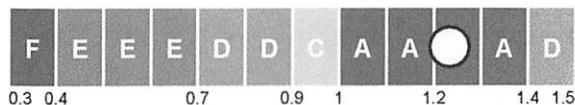
ABI test is a quick, noninvasive way to check your risk of peripheral artery disease (PAD). Peripheral artery disease is a condition in which the arteries in your legs or arms are narrowed or blocked.



Normal range

Left ANKLE/BRACHIAL INDEX (ABI) = 1.23

ABI test is a quick, noninvasive way to check your risk of peripheral artery disease (PAD). Peripheral artery disease is a condition in which the arteries in your legs or arms are narrowed or blocked.



Normal range

Right Toe/Brachial Index: 1.11

Determine the severity of peripheral arterial disease present in a lower extremity.

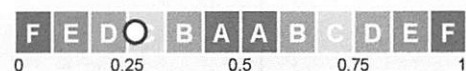
Normal range



Ejection Elasticity Index (EEI): 0.3

EEI is an indicator for left ventricle ejection power and elasticity of large arteries.

Possible reduced left ventricular ejection fraction / atherosclerosis. Possible causes: hypertension / hypercholesterolemia / obesity / metabolic syndrome / diabetes.



Dicrotic Dilation Index (DDI): 0.42

DDI indicates the contractility, tension and stiffness in the small arteries.

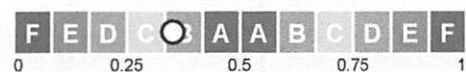
Normal Blood Circulation



Dicrotic Elasticity Index (DEI): 0.35

DEI represents the reflection of arterial elasticity and blood flow in the venous system.

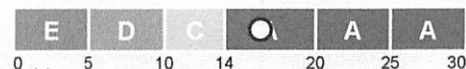
Borderline Normal Blood Circulation



Coronary Respiratory Response: 16.4

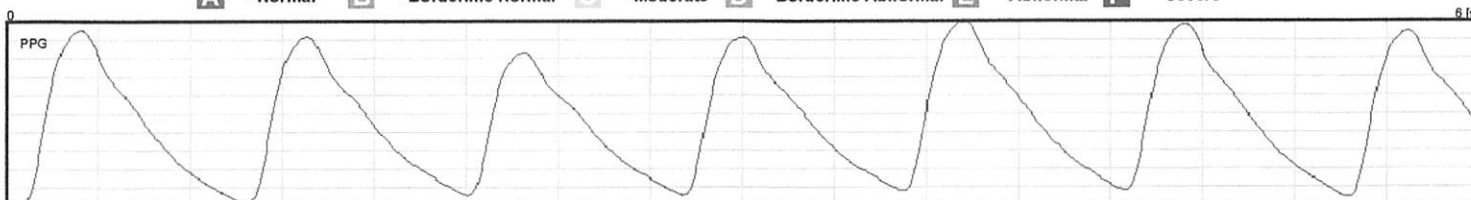
CAD occurs when the blood vessels that transport blood to the heart are narrowed and hardened due to plaque buildup (atherosclerosis).

Normal range



	Score	Units	Norms		Score	Units	Norms		Score	Units	Norms
Augmentation Index	0.26		< -0.7	Reflection Index	0.7		.65-.85	Stiffness Index	11.67	m/s	< 8.0
Stroke Volume	56	ml	55-100	Cardiac Output	3.69	l/min	4.0-8.0	Mean Arterial Pressure	84	mmHg	70-110
Blood Volume	3.75	l	3-5	C1	7.45	ml/mm...	> 10.0	C2	6.85	ml/mm...	> 6.0
DPTI/SPTI	0.58	ratio		Systemic Vascular Resistance	1798		700-18...	Pulse Oximetry	97.1		> 95
Blood Pressure	120/57	mmHg	< 120	Central Systolic Pressure	115	mmHg	102-124				

A - Normal **B** - Borderline Normal **C** - Moderate **D** - Borderline Abnormal **E** - Abnormal **F** - Severe



Physician's Notes:

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AUTONOMIC NERVOUS SYSTEM FUNCTION

Estella Jangam

Gender: Female

Age: 55 (DOB: Apr 19 1963)

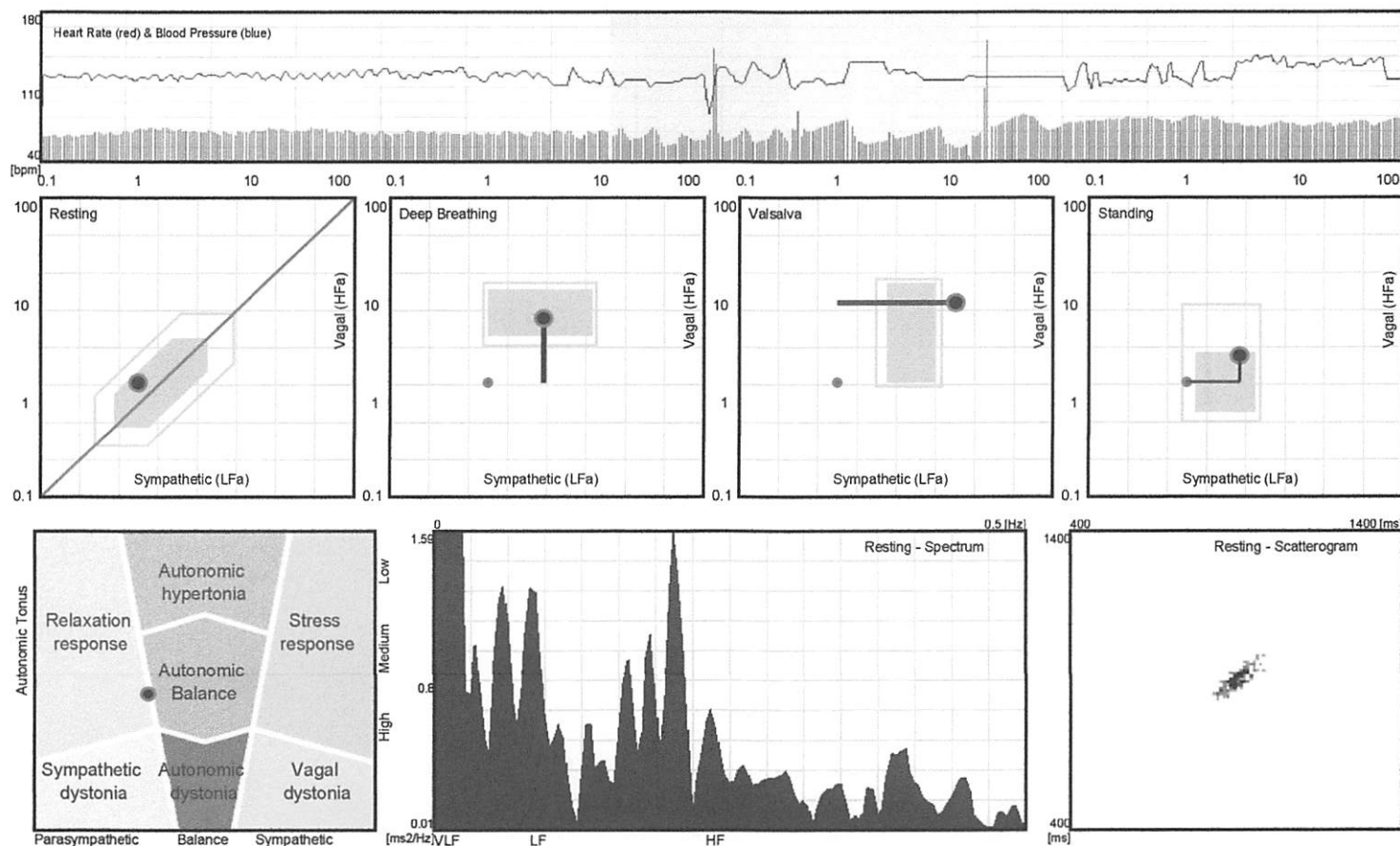
Weight: 145 lbs

BMI: 26.5

Height: 5 ft 2 in

Physician Only Report

Exam Date: Jun 12 2018 07:58



	HR	BP	RMF	LFa	HFa	LFa/HFa	LF/HF	TSP	SDNN	rmsSD	SpO2
Resting	66	120/57	0.23	0.86 N	1.41 N	0.61 L	1.11	1013	29	17	96
Deep Breathing	62	124/87	0.11	2.96	6.24 N	0.47	1.71	5641	69	45	98
Valsalva	64	117/79	0.11	11.69 H	8.94	1.31	1.51	8050	110	40	97
Standing	76	124/93	0.13	2.75 N	2.6 N	1.06	0.64	1812	45	34	93

Signal Quality = Resting: Good Deep Breathing: Good Valsalva: Good Standing: Good

Autonomic Tonus = 54.64 N E/I Ratio = 2.24 N Valsalva Ratio = 1.4 N 30:15 Ratio = 2.08 N

Deep Breathing: Normal Parasympathetic response to stimulation;

Valsalva: Hyperactive sympathetic response to stimulation, this could be cause for potential hypertension;

Standing: Normal ANS response to stimulation; Enhanced cardiovascular tolerance. The autonomic regulation of cardiovascular functions provides enormous ability to tolerate physical exertions and normal adaptation to their long-term effects. Typically this is a sign of an absence of physical exhaustion, overtraining or any cardiovascular health conditions. This is a typical pattern found in a physically active, healthy individual, without any evident signs of physical exhaustion, overtraining or other conditions altering the regulatory functions of the autonomic nervous system or the cardiovascular response to it;

Physician's Notes:

All results and analysis should be considered in the context of persons/candidate's case history, symptoms, diagnosis, current medications, treatment plans and therapies. Final diagnosis is the sole responsibility of the licensed medical practitioner after persons examination, lab tests and/or other clinical findings as necessary.

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AUTONOMIC BALANCE - STANDING

Estella Jangam

Gender: Female

Age: 55 (DOB: Apr 19 1963)

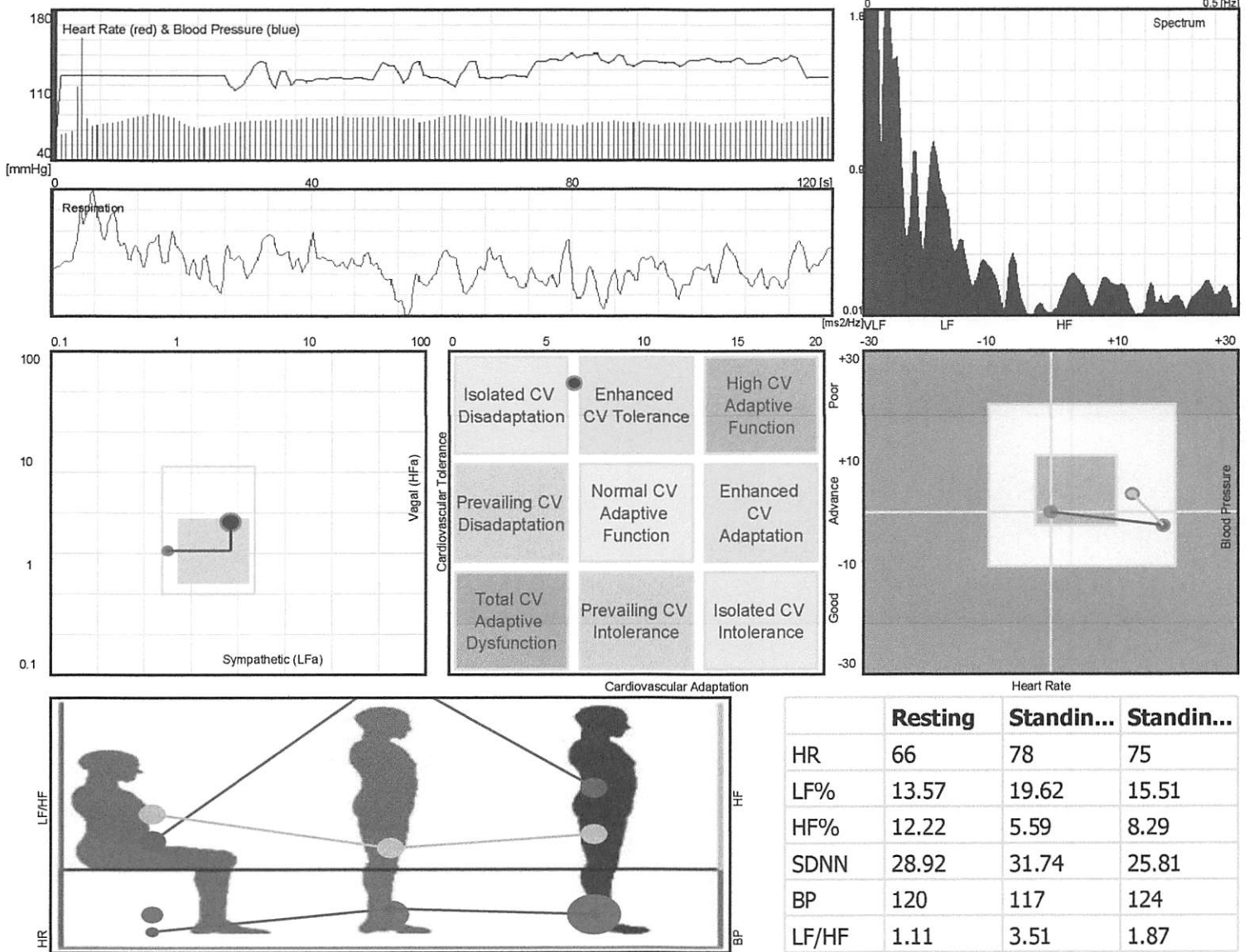
Weight: 145 lbs

BMI: 26.5

Height: 5 ft 2 in

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Exam Date: Jun 12 2018 07:58



Total HeartBeats = 151 Artifacts = 3 (2%) Signal Quality = Good

	value(re->st)	ratio(st/re)	normal	borderline	high	low
HFa	1.41->2.6	1.84	<2	2-6	>6	
LFa	0.86->2.75	3.19	1.2-4.5	4.5-5	>5	<0.9
LFa/HFa	0.61->1.06	1.73	1.2-5	0.9-1.2	>5	<0.9
HR	66->76	1.15	>1.1	1-1.1	>30	<1
30:15 Ratio	2.08		>1.1			<1.1
TSP	1811	LF/HF	0.64	RMF	0.13	
SDNN	45	rmsSD	34	pNN50	2	
SDSD	34.05	PH	31	SpO2	93	

HR,PD - bpm; RMF - Hz; VLF,LF,HF,TSP - ms²; HFa,LFa - bpm²

Normal ANS response to stimulation; Enhanced cardiovascular tolerance. The autonomic regulation of cardiovascular functions provides enormous ability to tolerate physical exertions and normal adaptation to their long-term effects. Typically this is a sign of an absence of physical exhaustion, overtraining or any cardiovascular health conditions. This is a typical pattern found in a physically active, healthy individual, without any evident signs of physical exhaustion, overtraining or other conditions altering the regulatory functions of the autonomic nervous system or the cardiovascular response to it.

Physician's Notes:

ECG ANALYSIS REPORT

Estella Jangam

Gender: Female

Age: 55 (DOB: Apr 19 1963)

Weight: 145 lbs

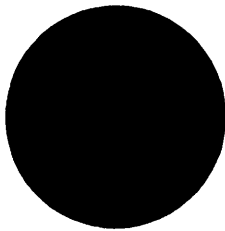
BMI: 26.5

Height: 5 ft 2 in

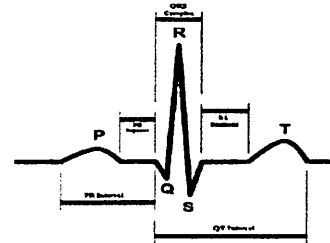
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Exam Date: Jun 12 2018 07:58

Recorded time		Heart Rate	
Start Date	2018-06-12 07:58:01	Average Heart Rate	66 bpm
Duration	3 min (200 beats)	Fastest rate	72 bpm
High Pass Filters	3.4 Hz	Slowest rate	61 bpm
Ventricular Details		Supraventricular Details	
PVC - Ventricular Ectopy	0 beats (0%)	PAC - Supraventricular Ectopy	0 beats (0%)
Ventricular Couplet	0 episodes	Supraventricular Couplet	0 episodes
HRV Analysis		QRS Analysis	
SDNN	21 ms	QRS	66 ms
		QT / QTc	288 ms / 304 ms
		PR int / seg	148 ms / 98 ms
		ST int / seg	211 ms / 107 ms

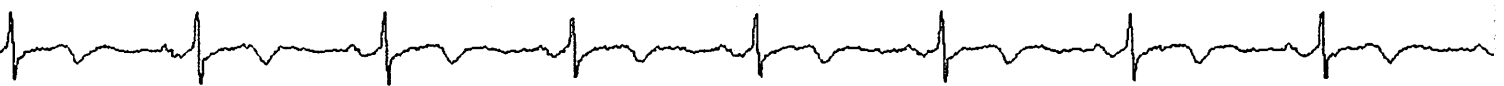


☒ PAC - Atrial Extrasystole: 0%
☒ PVC - Ventricular Extrasystole: 0%
☒ Normal: 100%

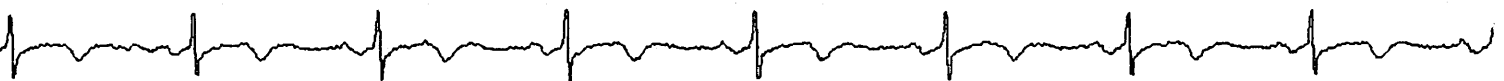


Interpretations:

1.5 945 N 950 N 955 N 960 N 965 N 970 N 975 N 7.0 [s]



-1.5 [mv] 950 N 955 N 960 N 965 N 970 N 975 N 15.2 [s]



-1.5 [mv]

Physician's Notes:

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Printed: Jun 12 2018 08:07

Arterial Stiffness Assessment - Pulse Wave Velocity

Estella Jangam

Gender: Female

Age: 55 (DOB: Apr 19 1963)

Weight: 145 lbs

BMI: 26.5

Height: 5 ft 2 in

Physician Only Report

Exam Date: Jun 12 2018 07:58

Ejection Elasticity Index (EEI) = 0.27

EEI is an indicator for left ventricle ejection power and elasticity of large arteries.



Possible reduced left ventricular ejection fraction / atherosclerosis. Possible causes: hypertension / hypercholesterolemia / obesity / metabolic syndrome / diabetes.

Dicrotic Dilation Index (DDI) = 0.42

DDI indicates the contractility, tension and stiffness in the small arteries.



Normal Blood Circulation

Dicrotic Elasticity Index (DEI) = 0.35

DEI represents the reflection of arterial elasticity and blood flow in the venous system.



Borderline Normal Blood Circulation

Augmentation Index (AI) = 0.26

Augmentation Index (AI) is a useful marker for cardiac risk. AI increases with age and a sedentary lifestyle.



AI is a measure of arterial stiffness and it provides general information about the arteries. AI is positively correlated with pulsewave velocity, blood pressure and hypertension.

A = Excellent
B = Borderline Normal
C =
D = Borderline Abnormal
E = Abnormal - Severe
F = Severe

Reflection Index (RI) = 0.65

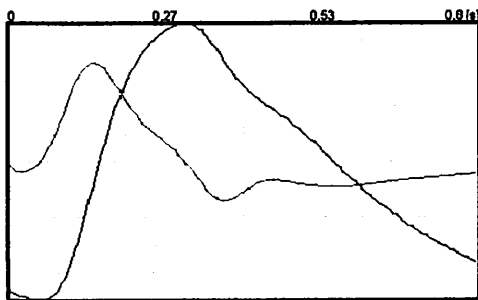


RI is an indicator of the vascular tone of the small arteries. Both vasodilation and vasoconstriction play important roles in determining vascular tone.

APG Pattern

An indication of the biological, (rather than chronological) age of arteries

APG Type = G



Stiffness Index (SI) = 11.7 m/s



SI is a measure of large artery stiffness determined by time. SI calculation gives a value similar to aortic pulse wave velocity.

Heart Rate = 66 (bpm) SpO2 = 96.8 (%)

C1 - Capacitive Arterial Compliance = 7.41 ml/mmHg

C2 - Oscillatory or Reflective Arterial Compliance = 6.77 ml/mmHg

Diastolic/Systolic Pressure Time Index(DPTI/SPTI) = 0.58

Systemic Vascular Resistance (SVR): 1798 (700-1800)

Blood Volume (BV): 3.75 l (3-5)

Cardiac Output (Q): 3.69 l/min (4.0-8.0)

Pulse Pressure (PP): 27.99 mmHg (25-100)

Physician's Notes:

SUDOMOTOR SYMPATHETIC RESPONSE

Estella Jangam

Gender: Female

Age: 55 (DOB: Apr 19 1963)

Weight: 145 lbs

BMI: 26.5

Height: 5 ft 2 in

Physician Only Report

Exam Date: Jun 12 2018 07:50

HANDS

Asymmetry = 14%

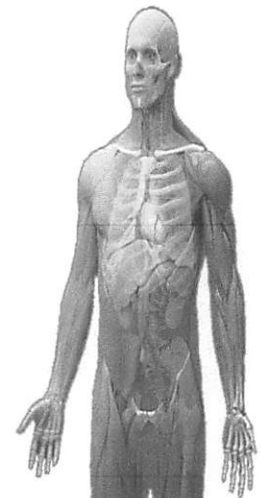
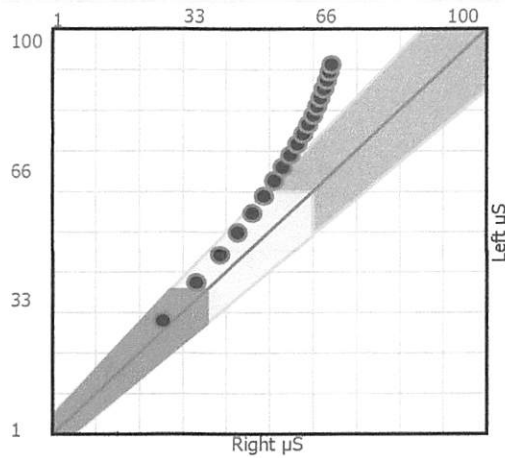
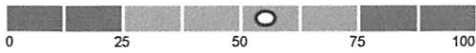
(Unfavorable values: higher than 20 %)

Bioelectrical Conductivity Response (dBEC) = 71

(dBEC correlate with Sweat Gland Function)

Bioelectrical Conductivity (BEC) = 55.2 μ S

(BEC correlate with Sweat Gland Nerve Density)



FEET

Asymmetry = 9%

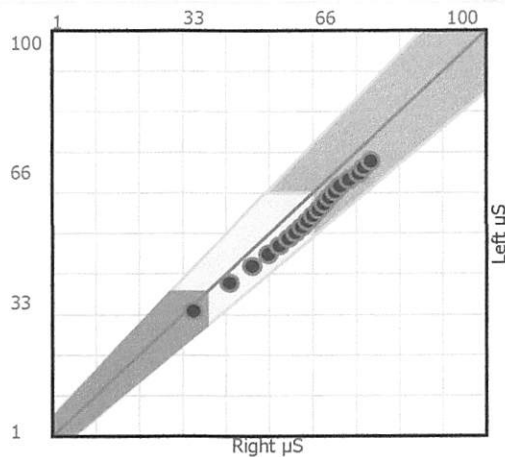
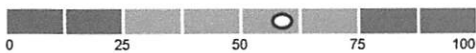
(Unfavorable values: higher than 20 %)

Bioelectrical Conductivity Response (dBEC) = 76

(dBEC correlate with Sweat Gland Function)

Bioelectrical Conductivity (BEC) = 58.7 μ S

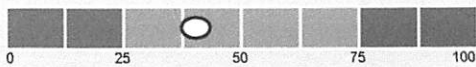
(BEC correlate with Sweat Gland Nerve Density)



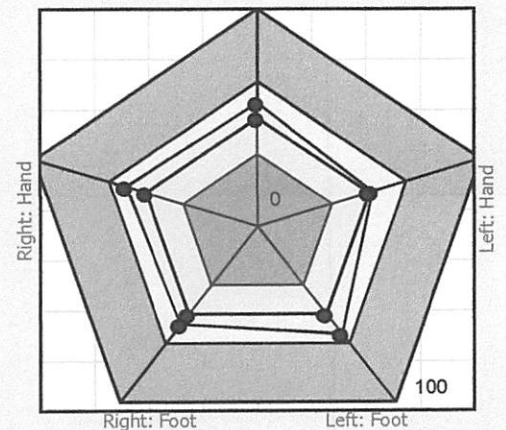
SUDOMOTOR AUTONOMIC NEUROPATHY

Risk for sudomotor autonomic neuropathy = 40.2

(No risk if < 40, Moderate 40-60, Elevated risk if > 60)



No clear sign of Sudomotor dysfunction.



Sudomotor autonomic neuropathy correlate with Sweat Gland Nerve Density and Sweat Gland Function

Physician's Notes:

METABOLIC REPORT

Estella Jangam

Gender: Female

Age: 55 (DOB: Apr 19 1963)

Weight: 145 lbs

BMI: 26.5

Height: 5 ft 2 in

Physician Only Report

Exam Date: Jun 12 2018 07:50

Ideal Body Weight = 110 Lbs

Real Body Weight = 145 Lbs

Basal Metabolic Rate (BMR) = 1322 cal

Total Daily Energy Expenditure = 1983 cal

 Normal

 Borderline Normal

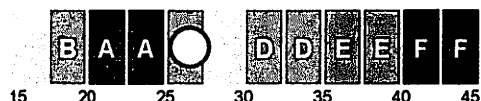
 Mild - Moderate

 Borderline Abnormal

 Abnormal - Severe

Body Mass Index (BMI) = 26.5

(Normal value range: 19 - 25)



Body mass index, or BMI, is a new term to many people.

However, it is the measurement of choice for many physicians and researchers and it is used to estimate a healthy body weight based on a person's height, assuming an average body composition.

It is the most widely used diagnostic tool to identify weight problems within a population.

Body mass index is defined as the individual's body weight divided by the square of his or her height.

The body mass index can be used to identify if you are overweight.

A drawback of the calculation is that if you are muscular it can suggest you are overweight due to muscle density.

An elevated BMI is associated with Metabolic Syndrome and is tied to an elevated risk of type 2 diabetes, hypertension, and cardiovascular disease.

Risk of Associated Disease According to BMI and Waist Size

18.5 or less: Underweight - N/A

19 - 25: Normal - very low risk of associated diseases

26 - 29: Overweight - prone to health risks

30 - 40: Overweight to Obese - high risk of associated diseases

40 or greater: Extremely Obese - very high risk of associated diseases

The Basal Metabolic Rate (BMR) shows the calories (energy) your body uses per day while at rest.

The Total Daily Energy Expenditure shows the calories needed to maintain your current weight.

For healthy weight management increase your caloric usage (exercise) and decrease your caloric intake below the Total Daily Energy Expenditure towards the Basal Metabolic Rate (BMR).

Eating a high quality, nutrient dense diet (fresh vegetables (cooked and raw), chicken, fish, eggs, and yogurt) and staying away from carbohydrates and poor quality fats helps to prevent cravings and aids in weight loss.

If you go too far below the Basal Metabolic Rate (BMR) your metabolism may slow down making weight management more difficult.

Physician's Notes: